Supreme Court, U.S.
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IN THE
SUPREME COURT OF THE UNITED STATES

October Term, 1997

JANE M. ROBERTS, Guardian for WANDA Y. JOHNSON — **Petitioner**

versus

GALEN OF VIRGINIA, INC., formerly d/b/a HUMANA HOSPITAL-UNIVERSITY OF LOUISVILLE, d/b/a UNIVERSITY OF LOUISVILLE HOSPITAL — **Respondent**

PETITION FOR WRIT OF CERTIORARI TO THE UNITED STATES COURT OF APPEALS FOR THE SIXTH CIRCUIT

PETITION FOR WRIT OF CERTIORARI

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89 pp

QUESTIONS PRESENTED

1. Whether a claim under the Emergency Medical Treatment and Active Labor Act, 42 U.S.C. § 1395dd requires proof of an improper motive on the part of the hospital, its staff or physicians as a pre-requisite for recovery?

2. Whether, if proof of an improper motive is a pre-requisite for recovery under the Emergency Medical Treatment and Active Labor Act, 42 U.S.C. § 1395dd, proof of improper motive on the part of hospital administration and social services staff involved in a patient's treatment and discharge is sufficient without additional proof that independent contractor treating physician who executed a patient's discharge order was also improperly motivated?

3. Whether, under Kentucky law, surgical residents not employed directly by a hospital, are the hospital's ostensible agents whose negligence can be attributed to the hospital?

LIST OF PARTIES

The caption of the case in this Court contains the names of all parties to the proceedings in the United States Court of Appeals for the Sixth Circuit.

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PETITION FOR WRIT OF CERTIORARI

The Petitioner, Jane M. Roberts, as Guardian for Wanda Y. Johnson, respectfully prays that a Writ of Certiorari issue to review the opinion and order of the United States Court of Appeals for the Sixth Circuit, entered on April 9, 1997.

OPINIONS BELOW

The opinion of the United States Court of Appeals for the Sixth Circuit, whose judgment is herein sought to be reviewed, is reported at 111 F.3d. 405 (6th Cir. 1997) and is reprinted in the appendix hereto, pp. A1-A25. The two prior opinions of the United States District Court for the Western District of Kentucky in this case were not recommended for full-text publication but are reprinted in the appendix hereto, pp. A26-A45 and pp. A48-A52, respectively.

JURISDICTION

An order of the United States Court of Appeals for the Sixth Circuit was entered on April 9, 1997 affirming the order of the United States District Court for the Western District of Kentucky which granted summary judgment in favor of the respondent. The jurisdiction of this Court is invoked pursuant to 28 U.S.C. § 1254(1).

STATUTES INVOLVED

The statute involved in this case is the "Emergency Medical Treatment and Active Labor Act," 42 U.S.C. § 1395dd, reprinted in the appendix hereto, pp. A55-A65.

STATEMENT OF THE CASE

This action was filed in the United States District Court for the Western District of Kentucky on August 30, 1993 alleging, *inter alia*, that the Respondent, Galen of Virginia, Inc., formerly d/b/a Humana Hospital-University of Louisville, d/b/a University of Louisville Hospital (hereinafter "Humana") had violated the terms of 42 U.S.C. § 1395dd, The Emergency Medical Treatment and Active Labor Act (hereinafter "EMTALA") by transferring the Petitioner, Wanda Y. Johnson (hereinafter "Johnson"), to a nursing home in Indianapolis, Indiana before Humana had stabilized Johnson's condition. Johnson, a 36 year old mother of one child, was run over by a truck while walking home from her job as a cashier for Hardees Restaurant in Bardstown, Kentucky, late on the evening of May 19, 1992. She was flown from Bardstown to Humana where she was diagnosed with a left pelvic fracture, right fibular head fracture, T-4 and L3-L5 fractures and a closed head injury. Johnson underwent exploratory surgery upon admission to Humana during which her spleen was removed. She was placed on a mechanical ventilator and tracheostomy. During her hospitalization, she developed multiple pneumonias and diffuse gastritis. Her hospital course was further complicated by fever and sepsis with an elevated temperature to 108 degrees, the origin of which was never accurately determined. Following the temperature spike, Johnson's neurologic status deteriorated to a state of flaccid quadriparesis. Despite Humana's treatment,

Johnson's temperature spikes could not be controlled and episodes of pneumonia and urinary tract infection continued through the last week of her hospitalization.

In early July, 1992, Johnson's physician requested that the social services department at Humana begin making contacts with rehabilitation facilities and nursing homes to explore potential placement for Johnson if and when her condition was stabilized and she was ready to leave an acute care setting.

Humana's social workers immediately experienced difficulty in their efforts to find potential placement facilities for Johnson. The social workers learned that Johnson's husband had abandoned Johnson and their five year old son and had begun cohabitation with another woman in Springfield, Kentucky. No other close family member existed to provide assistance to the Humana social workers in developing a discharge plan for Johnson. Jane Roberts, Johnson's aunt, occasionally visited the hospital and attempted to locate Johnson's husband for the social workers. However, Roberts had not been appointed as Johnson's guardian at that time and did not have the legal authority, nor had she been requested, to act as Johnson's surrogate decision maker.

Another significant complication in locating a facility which would accept Johnson was her inability to pay. By mid-July, 1992, it became evident that Johnson's application for Kentucky Medicaid benefits would not be approved. Johnson had no private health insurance and obviously did not

have the resources to compensate any facility for her care. Johnson's medical expenses at Humana had already approached $400,000.00 and it was obvious to Humana that it would never be compensated for those charges.

In the midst of Humana's placement dilemma, Joanna Nolte, a marketer for Hometown Living Centers ("Hometown") in Indianapolis, Indiana, visited Louisville in an effort to initiate a relationship with the Humana hospitals whereby Humana would place its patients in Hometown's Indianapolis nursing homes.

On the evening of July 22, 1992, Nolte met with members of Humana's Social Services Department. Among those who attended Nolte's presentation was Nancy Fred, who had taken charge of Johnson's discharge plan. At the conclusion of Nolte's presentation, she was approached by Nancy Fred. At that time, Fred requested that Nolte look at Johnson and consider accepting Johnson in one of the Hometown facilities. According to Nolte, Fred appeared to be in a real placement dilemma with Johnson. Nolte testified that Fred advised her that she was receiving pressure from the administration at Humana to find a placement for Johnson because of Johnson's inability to pay for her care. Nolte explained to Fred that Johnson's level of acuity exceeded Nolte's ability to grant approval and indicated that she would have to refer Fred's request to the Administrator and Director of Nursing at Mayfair Manor for evaluation.

All the while, Johnson's medical condition continued to be volatile. On July 23, 1992, her temperature spiked to

101 degrees. A sputum culture revealed positive cocci and her doctor suspected that she had yet another episode of urinary tract infection. On the evening of July 23, 1992, Johnson's right lung collapsed and treatment was required every two hours. On July 24, 1992, the date of Johnson's discharge, her white blood count was over twice the normal level, her temperature had been rising for two days, and the nurses noticed cloudy urine, thick yellow sputum and an elevated heart rate. Karen Martin, the nurse on duty during Johnson's last day of hospitalization at Humana, testified that all of these symptoms indicated the presence of infection.

Meanwhile, Mayfair Manor declined Humana's referral of Johnson because her care required too many skilled nursing hours. Yet, somehow another Hometown facility, Crestview Nursing Home ("Crestview"), which had even fewer skilled nurses than Mayfair, picked up the Johnson referral and agreed to accept her. The first notation that Crestview had even been considered by the social workers at Humana appears in a social services note on July 24, 1992, wherein Nancy Fred indicated that Johnson would be transferred to Crestview on that date at 2:30 p.m.

In its haste to expedite Johnson's transfer to Crestview, Humana ignored its standard procedures and protocols for discharge and transfer. No authorization for the transfer was obtained from Johnson's husband or mother. Nancy Fred simply called Jane Roberts, Johnson's aunt and occasional visitor, and *told her* that Johnson would be transferred to Crestview Nursing Home in Indianapolis that

afternoon. Fred gave Roberts Crestview's telephone number. She did not ask Roberts to check with Johnson's family to determine whether they would like to visit the facility before the transfer was arranged or whether they would even approve of a transfer which would place Johnson some three hours driving distance from their home. Fred gave Roberts no information about Crestview or about any benefits it could provide to Johnson. In fact, neither Fred nor anyone else at Humana could then provide any such information because no one knew much about Crestview.

In addition to Humana's violation of its own discharge policy and protocols which mandate advising the patient or her family of discharge plans in order to provide the patient or family at least an opportunity to decline the discharge and make other arrangements, Humana also violated its policy and protocol by failing to gain approval for the transfer from the accepting physician at Crestview. The physicians for Crestview were unaware of Johnson's transfer until she had already arrived.

There can be little question that Crestview was not an appropriate placement for Johnson. Even the Crestview physicians testified that its facility was inappropriate.

Johnson's condition deteriorated significantly as a result of her transfer from Humana. The nurses at Crestview were so alarmed by Johnson's condition upon her arrival in Indianapolis that they thought she was going to die. Within hours of her arrival at Crestview, Johnson's temperature had spiked to 105 degrees and she was rushed to Midwest

Medical Center where she remained hospitalized for almost six months. Dr. John W. Stuy was Johnson's principal treating physician during her hospitalization in Indianapolis and he testified that the complications experienced by Johnson and which caused her deterioration were present while she was hospitalized in Louisville but were exacerbated because she was transferred from Humana to a facility which was unable to properly care for her. Dr. Stuy testified that the antibiotic that was prescribed for Johnson when she left Humana was not sensitive to the cultures which were grown at Midwest Medical Center and was not treating her infection. He commented that if she had remained on the antibiotic much longer, she may have died. Dr. Stuy testified that Johnson should never have been transferred from Humana.

As a final insult, Humana's transfer of Johnson to Indianapolis placed her in a jurisdiction where she was ineligible for public medical assistance. Had she remained hospitalized at Humana, Johnson would never have incurred personal liability for the payment of medical expenses because of Humana's receipt of millions of dollars from Kentucky governmental entities to provide care to indigent patients. However, once Johnson was transferred from Humana to Indianapolis she was no longer entitled to that benefit. In fact, Johnson's application for Medicaid benefits in Indiana was denied because she was not a resident of Indiana. As a direct result of Humana's unilateral decision to ship Johnson to Indiana, Johnson incurred medical expenses of over $300,000.00 for which she is now personally

obligated.

Although the District Court had initially determined that genuine issues of material fact existed which precluded Summary Judgment in favor of Humana on the EMTALA claim, upon reconsideration it determined that proof of improper motive was a prerequisite for recovery under EMTALA. It then found that even though Johnson had presented proof that could indicate that Humana's staff and administration was improperly motivated in arranging Johnson's transfer, because Johnson had not provided proof that the independent contractor physicians who signed the order to discharge her had also been improperly motivated, no genuine issue of material fact existed as to "causation" and therefore granted Humana Summary Judgment on Johnson's EMTALA claim and dismissed her case.

On appeal, Johnson argued that every other Circuit Court of Appeals in the country which had considered the issue had expressly rejected the notion that proof of improper motive was required to establish a claim under EMTALA. However, in a two to one decision, the Court of Appeals for the Sixth Circuit split with the interpretation of EMTALA by the other Circuit Courts of Appeals and held that EMTALA required proof of improper motive. It further held that proof of improper motive by hospital administration is insufficient and required proof that the discharging physician was improperly motivated for a claimant to succeed under EMTALA.

REASON FOR GRANTING THE WRIT

Certiorari Should Be Granted To Resolve Conflicts Among the Lower Courts in the Interpretation of a Federal Statute.

The principal issue in this case is whether a claim under EMTALA requires proof of an improper motive on the part of the hospital, its staff or physicians as a prerequisite for recovery. The issue is a substantial one to every potential claimant under EMTALA. In fact, if a claimant must prove the inner thoughts and prejudices of attending hospital personnel in order to recover under EMTALA, the successful pursuit of a civil EMTALA claim is virtually impossible.

If certiorari were granted, this would be the first case before this Court interpreting the prerequisites to recovery under EMTALA. This Court could then provide direction to the lower courts in an area where there has developed a clear split in interpretation of the relevant federal statute.

The various Circuit Courts of Appeals have struggled to interpret EMTALA as providing a substantial federal remedy while attempting to provide some limits on the application of the statute so that EMTALA is not utilized as a federal medical malpractice statute. The Circuit Courts of Appeals have uniformly held that, "EMTALA is not a substitute for state law malpractice actions, and was not intended to guarantee proper diagnosis or to provide a federal remedy for misdiagnosis or medical negligence." *Power v.*

Arlington Hosp. Ass'n, 42 F.3d 851, 856 (4th Cir. 1994). *See also Roberts v. Galen of Virginia, Inc.*, 111 F.3d 405 (6th Cir. 1997); *Summers v. Baptist Medical Ctr. Arkadelphia*, 91 F.3d 1132, 1137 (8th Cir. 1996); *Correa v. Hospital San Francisco*, 69 F.3d 1184, 1192 (1st Cir. 1995), *cert. denied*, ___ U.S. _____, 116 S. Ct. 1423, 134 L.Ed.2d 547 (1996); *Eberhardt v. City of Los Angeles*, 62 F.3d 1253, 1258 (9th Cir. 1995); *Repp v. Anadarko Mun. Hosp.*, 43 F.3d 519, 522 (10th Cir. 1994); *Holcomb v. Monahan*, 30 F.3d 116, 117 (11th Cir. 1994).

The Courts have labored to define exactly what a claimant must prove if the statute does not create a federal remedy for medical malpractice. The Court of Appeals for the Sixth Circuit determined that the way to limit the potentially sweeping scope of the statute's language is to require that a claimant prove improper motive in order to recover under EMTALA. *Roberts v. Galen of Virginia, Inc., supra.*; *Cleland v. Bronson Health Care Group, Inc.*, 917 F.2d 266 (6th Cir. 1990). While the Sixth Circuit's interpretation clearly prevents EMTALA from being utilized as a substitute for a state law malpractice action, in the opinion of the Fourth Circuit such an interpretation also makes proving a civil EMTALA claim virtually impossible. *Power v. Arlington Hosp. Ass'n, supra.* The Fourth Circuit explained its rejection of the Sixth Circuit's reasoning as follows:

> First, there is nothing in the statute itself that requires indigence, inability to pay or any other improper motive on the part of a hospital as a prerequisite to recovery. The language of § 1395dd(a) simply refers to "any individual" who presents to the emergency room. Second, it seems to us that the expanse of motives suggested by the Sixth Circuit . . . is so broad as to be no limit at all, and as a practical matter amounts to not having a motive requirement. Anyone who alleges that she did not receive an appropriate medical screening examination can simply find an improper motive that fits, whether it is sex, nationality, income, or occupation, and simply allege it. Which leads, thirdly, to the most fundamental problem with the motive requirement: the proof predicament. We agree . . . that having to prove the existence of an improper motive on the part of a hospital, its employees or its physicians, would make a civil EMTALA claim virtually impossible. We do not believe that proving the inner thoughts and prejudices of attending hospital personnel is required in order to recover under EMTALA.

Power v. Arlington Hosp. Ass'n, 42 F.3d, at 857-858.

Based on this or similar reasoning, the Courts of Appeal for the District of Columbia Circuit, the First Circuit, the Fourth Circuit and the Eighth Circuit expressly disagree that proof of an improper motive is a prerequisite to recovery under EMTALA. *Gatewood v. Washington Health Care Corp.*, 933 F.2d 1037 (D.C. Cir. 1991); *Correa v. Hospital San Francisco, supra.*; *Power v. Arlington Hosp. Ass'n, supra.*; *Summers v. Baptist Medical Ctr. Arkadelphia, supra.* Rather than adopting the limitation espoused by the Sixth Circuit, each of the other Circuit Courts of Appeal have determined that the proof required of a claimant in order to succeed under EMTALA consists of evidence to show that the claimant was not treated as other similarly situated patients are treated, within the hospital's capabilities. According to each of the Circuit Courts of Appeal that have considered the issue, other than the Sixth, it is up to the hospital itself to establish its own policies and procedures. Having done so, the Hospital must apply them alike to all patients.

The significance to a claimant of a uniform interpretation of the prerequisites to recovery under EMTALA could not be more clearly evident than it is with the facts of the case at bar. Both the District Court and the Court of Appeals in this case held that a genuine issue of fact existed as to whether or not Humana had stabilized Johnson's emergency medical condition prior to her discharge and transfer. Furthermore, Johnson was able to produce substantial evidence that she was not treated as other similarly

situated patients are treated at Humana. Specifically, Johnson produced Humana's own policies and procedures which require that when a patient is transferred from the hospital's critical care complex to another facility, Humana's social services staff obtain approval for the transfer from the patient's family and further require the attending physician at Humana to obtain the prior approval of the physician at the receiving facility before discharging and transferring a patient.

These requirements were not simply ministerial in nature but went to the very heart of the discharge and transfer procedure at Humana. In fact, Johnson's most fundamental argument in this case is that had *anyone* acting with Johnson's best interest at heart been given an opportunity to investigate Crestview as a potential placement they would have readily recognized that Crestview could not handle Johnson's care, would have refused transfer to Crestview and would have insisted that other options be pursued. It makes sense that after a transfer decision has been made for a patient coming out of the critical care complex one of the most important functions of the physician is the arrangement of physician acceptance of the patient at the receiving facility. It is certainly probable that patient's coming from a critical care complex have special needs which require one-to-one consultation between the physician arranging the transfer and the physician accepting the patient at the receiving facility. Where, as in Johnson's case, the transferring physician knows nothing about the receiving facility, one-to-one consultation with the physician at the receiving facility is

imperative so that the transferring physician can determine that the receiving facility is capable of handling the patient's care needs. Had Humana physicians or social workers simply followed their own policies and procedures, this whole litigation would have been avoided. The litigation would have been avoided because the physicians responsible for accepting patients at Crestview would have declined acceptance of Johnson.

This proof of disparate treatment is sufficient in every Circuit in the United States, other than the Sixth, to get an EMTALA claim to a jury. However, in the Sixth Circuit no matter how compelling the other evidence, if the discharging physician cannot be implicated with a showing of improper motive in the execution of his or her discharge of a patient, a claim under EMTALA will fail.

The prerequisites for recovery under EMTALA should be uniform throughout the United States. Claimants in the Sixth Circuit should not be required to prove more, and cannot expect that they can prove less, than claimants in other areas of the country. Potential claimants in Circuits which have not directly addressed the issue should not be left to guess whether their Circuit will adopt the interpretation of the Sixth Circuit or the interpretation of the other Circuits. This clear divergence in the interpretation of this important federal statute among the Circuit Courts of Appeal is a compelling reason for this Court to issue a Writ of Certiorari in this case.

CONCLUSION

For the foregoing reasons, this Petition for Certiorari should be granted and consider those issues identified herein.

Respectfully submitted on
July 3, 1997,
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APPENDIX

No. 96-5298

UNITED STATES COURT OF APPEALS
FOR THE SIXTH CIRCUIT

JANE ROBERTS, as Guardian for
Wanda Y. Johnson,
Plaintiff-Appellant,

v.

GALEN OF VIRGINIA, INC.,
formerly d/b/a Humana Hospital-
University of Louisville, d/b/a
University of Louisville Hospital,
Defendant-Appellee.

Decided and Filed April 9, 1997

Before: KENNEDY, NELSON, and VAN GRAAFEILAND, Circuit Judge.

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* The Honorable Ellsworth A. Van Graafeiland, Circuit Judge of the United States Court of Appeals for the Second Circuit, sitting by designation.

KENNEDY, J., delivered the opinion of the court, in which VAN GRAAFEILAND, J., joined. NELSON, J. (pp. 18-89), delivered a separate opinion concurring in part and dissenting in part.

KENNEDY, Circuit Judge. Plaintiff, Jane Roberts, appeals the District Court's order granting summary judgment for the defendant in this action under the Emergency Medical Treatment and Active Labor Act, 42 U.S.C. § 1395dd (Supp. 1996). Plaintiff alleges that her critically-ill niece, Wanda Johnson, was transferred from the defendant hospital to a nursing care facility in an unstable condition in violation of the Emergency Medical Treatment and Active Labor Act. For the reasons set forth below, we **AFFIRM** the judgment of the District Court.

I.

Wanda Johnson was hospitalized at Humana Hospital-University of Louisville ("Humana") on May 19, 1992 after sustaining massive severe injuries caused by a collision with a truck. A few weeks before July 22, 1992, the surgical resident treating Johnson requested a Humana social worker to search for a long term care facility for Johnson. While the social worker, Nancy Fred, was aware Johnson had no insurance, the resident who authorized her discharge had no such knowledge. On July 22, 1992, after two nursing homes turned Johnson down following on-site evaluations of her condition by their directors of nursing, a representative of Crestview Health Care Facility in the State of Indiana agreed

to take Johnson. She was transferred to Crestview on July 24, 1992.

The day after Johnson's arrival at Crestview, Johnson's condition deteriorated significantly. She was taken to Midwest Medical Center, also in Indiana, where she remained for many months incurring $388,679.93 in medical bills. The State of Indiana rejected Johnson's application for medical assistance under the Indiana Medicaid Program because she failed to meet the Indiana residency requirements.

On August 30, 1993, plaintiff, as guardian for Wanda Johnson, filed this action under the Emergency Medical Treatment and Active Labor Act ("EMTALA"), 42 U.S.C. § 1395dd (Supp. 1996), alleging that Humana failed to comply with EMTALA in the transfer of Johnson to a skilled nursing facility. Plaintiff also alleged in her complaint that Humana violated the Due Process and Equal Protection Clauses of the United States and Kentucky Constitutions and that Humana should be liable for negligence under state law for the acts of the medical residents responsible for Johnson's care. Humana filed a motion for summary judgment on all of plaintiff's claims.

The District Court initially denied Humana's motion for summary judgment on the EMTALA claim because it found there was a genuine issue of material fact as to whether Johnson was stabilized before she was transferred to Crestview and because there was a genuine issue of material

fact as to whether Humana acted with an improper motive. The District Court granted Humana's motion on the remaining claims. Regarding the state law negligence claim, the District Court held that the hospital could not be liable for the actions of its residents because the residents were not ostensible agents of the hospital. Humana moved for reconsideration of the EMTALA claim. On reconsideration, the District Court held that, despite the existence of genuine issues of material fact as to stabilization and improper motive, plaintiff failed to prove that "either the medical opinion that Johnson was stable or the decision to authorize her transfer was caused by an improper motive." Thus, on reconsideration, the District Court granted Humana's motion for summary judgment on the EMTALA claim. Plaintiff appeals solely from the disposition of the EMTALA claim and the District Court's conclusion that the residents were not ostensible agents of the hospital.

II.

This Court's review of a grant of summary judgment is *de novo*; we use the same test used by the District Court. *See Brooks v. American Broadcasting Cos.*, 932 F.2d 495, 500 (6th Cir. 1991). In reviewing summary judgment motions, courts must view the evidence in the light most favorable to the nonmoving party to determine whether a genuine issue of material fact exists. *See Adickes v. S.H. Kress & Co.*, 398 U.S. 144, 157 (1970). Under Fed. R. Civ. P. 56(c), summary judgment is proper if the evidence

"'show[s] that there is no genuine issue as to any material fact and that the moving party is entitled to [a] judgment as a matter of law.'" *Canderm Pharmacal, Ltd. V. Elder Pharmaceuticals, Inc.*, 862 F.2d 597, 601 (6th Cir. 1988) (quoting Fed. R. Civ. P. 56(c)).

III.

A.

The Emergency Medical Treatment and Labor Act, 42 U.S.C. § 1395dd, in pertinent part, provides:

> If any individual (whether or not eligible for benefits under this subchapter) comes to a hospital and the hospital determines that the individual has an emergency medical condition, the hospital must provide either--
>
> (A) within the staff and facilities available at the hospital. for such further medical examination and such treatment as may be required to stabilize the medical condition, or
>
> (B) for transfer of the individual to another medical

> facility in accordance with subsection (c) of this section.

42 U.S.C. § 1395dd(b)(1). As defined by the EMTALA, "to stabilize" under subsection (b)(1)(A) means, "to provide such medical treatment of the condition as may be necessary to assure, within reasonable medical probability, that no material deterioration of the condition is likely to result from or occur during the transfer of the individual from a facility . . . " 42 U.S.C. § 1395dd(e)(3)(A). As explained by our Court in *Cleland v. Bronson Health Care Group, Inc.*, 917 F.2d 266 (6th Cir. 1990):

> the impetus to this legislation came from highly publicized incidents where hospital emergency rooms[1] allegedly, based only on a patient's financial inadequacy, failed to provide a medical screening that would have been provided a paying patient, or transferred or discharged a patient without taking steps that would have been taken for a paying patient.

[1]As concluded by our Court in *Thorton v. Southwest Detroit Hospital*, 895 F.2d 1131 (6th Cir. 1990), EMTALA's application is not limited to a patient's treatment and discharge from an emergency room. Rather, "once a patient is found to suffer from an emergency medical condition in the emergency room, she cannot be discharged until the condition is stabilized, regardless of whether the patient stays in the emergency room." *Id.* at 1134.

Id. at 268. Although the legislative history suggests otherwise, the EMTALA applies to all patients regardless of their financial status. *Id.*

The dispute in this case arises out of the District Court's conclusion that our Court's decision in *Cleland* requires that plaintiff prove the hospital acted with an improper motive in order to recover under EMTALA. Specifically, the District Court concluded and the defendant contends that merely transferring a patient before the patient is sufficiently stabilized does not constitute a violation of EMTALA; rather, plaintiff must prove that the hospital acted with an improper motive when it failed to stabilize the patient before transfer. In *Cleland*, parents of a fifteen-year-old boy took their soon to an emergency room complaining of cramps and vomiting. A doctor at the hospital examined the boy, diagnosed influenza, and discharged the patient four hours later. *Id.* at 268. Less than twenty-four hours after he was discharged, the boy died of cardiac arrest caused by intussusception.[2] The district court dismissed the parents' cause of action under EMTALA because the court concluded that the EMTALA applied only to indigent patients. This Court affirmed the dismissal of the complaint, but on different grounds. *Id.*

In analyzing whether the Clelands' claim was properly dismissed, this Court first noted that the parents did not allege

[2]The slipping of one part of an intestine into another part just below it. TABER'S CYCLOPEDIC MEDICAL DICTIONARY I-35 (9th ed. 1963).

that the care provided to their son was deficient or that the hospital's actions in connection with their son's screening and stabilization was in any way different than the care that would have been offered to any other patient. *Id.* at 269. Our Court then sought to define the terms "appropriate medical screening" and "emergency medical condition" which appear in subsection (a) of EMTALA in order to determine whether the Clelands' claim could survive despite their admission that their son's treatment was not disparate or insufficient but simply wrong. Subsection (a) of EMTALA states:

> In the case of a hospital that has a hospital emergency department, if any individual . . . comes to the emergency department and a request is made on the individual's behalf for examination or treatment for a medical condition, the hospital must provide for an *appropriate medical screening examination* within the capability of the hospital's emergency department, including ancillary services routinely available to the emergency department, to determine whether or not an *emergency medical condition* . . . exists.

42 U.S.C. § 1395dd(a)(emphasis added). The court refused to interpret the term "appropriate" as referring to a malpractice or other standard of care. Instead, we concluded

that the term "appropriate" refers "to the motives with which the hospital acts." *Cleland*, 917 F.2d at 272. The court further explained, "If it acts in the same manner as it would have for the usual paying patient, then the screening provided is 'appropriate' within the meaning of the statute." *Id.*

Addressing the concern that this approach might "constitute a backdoor means of limiting coverage to the indigent or uninsured," this Court commented:

> . . . A hospital that provides a substandard (by its standards) or nonexistent medical screening for any reason (including, without limitation, race, sex, politics, occupation, education, personal prejudice, drunkenness, spite, etc.) may be liable under this section. Similarly, a discharge that to the knowledge of those conducting it left a patient with an "emergency medical condition" in an "unstable" condition would be actionable.
>
> We can think of many reasons other than indigency that might lead a hospital to give less than standard attention to a person who arrives at the emergency room. These might include: prejudice against the race, sex, or ethnic group of the patient; distaste for the patient's condition (e.g., AIDS patients); personal dislike or antagonism between the

> medical personnel and the patient; disapproval of the patient's occupation; or political or cultural opposition. If a hospital refused treatment to a person for any of these reasons, or gave cursory treatment . . . the patient would fall squarely within the statutory language.

Id. Because the parents did not allege that their son was treated differently than any other patient, we affirmed the district court's dismissal of their claim. *Id.*

The District Court in the instant case properly interpreted the *Cleland* holding as requiring that a plaintiff prove a hospital acted with an improper motive in order to recover under the EMTALA. As noted by the United States Court of Appeals for the Fourth Circuit, "EMTALA is not a substitute for state law malpractice actions, and was not intended to guarantee proper diagnosis or to provide a federal remedy for misdiagnosis or medical negligence." *Power v. Arlington Hosp. Ass'n*, 42 F.3d 851, 856 (4th Cir. 1994). *See also Summers v. Baptist Medical Ctr. Arkadelphia*, 91 F.3d 1132, 1137 (8th Cir. 1996); *Correa v. Hospital San Francisco*, 69 F.3d 1184, 1192 (1st Cir. 1995), *cert. denied*, 116 S. Ct. 1423 (1996); *Eberhardt v. City of Los Angeles*, 62 F.3d 1253, 1258 (9th Cir. 1995); *Repp v. Anadarko Mun. Hosp.*, 43 F.3d 519, 522 (10th Cir. 1994); *Holcomb v. Monahan*, 30 F.3d 116, 117 (11th Cir. 1994). To distinguish

an EMTALA claim from a state law claim for negligence, a plaintiff must establish something more than a hospital's breach of the applicable standard of care. We reject the position espoused by plaintiff that, to succeed on a claim under the EMTALA, she can prove that Humana's treatment of Johnson was not uniform to patients suffering from the same medical condition as Johnson. Proof of disparate treatment, in plaintiff's view, distinguishes an EMTALA claim from a state law negligence claim. If we were to adopt this position, however, we would effectively require a hospital, in defense of a claim under EMTALA, to either prove that it breached a standard of care to an individual patient or that it breaches the applicable standard of care with respect to all similarly situated patients. In other words, we would require a hospital to prove that it has committed medical negligence or malpractice. In order to prevent placing hospitals in such a precarious position, we declined in *Cleland* to adopt such an approach. Instead, *Cleland* imposed upon the plaintiff a burden to bring forth some showing of improper motivation. As explained in *Cleland*, a plaintiff is not limited to a showing of improper motivation involving indigency or lack or insurance. Rather, other improper reasons include race, sex, politics, occupation, education, personal prejudice, drunkenness, or spite; that is, anything except medical negligence. *Cleland* requires proof of the existence of any improper motivation such as those listed above to make a case of inappropriate screening or

improper discharge.[3] To interpret *Cleland* in any other manner would effectively reduce the EMTALA to nothing more than a federal remedy for medical malpractice.[4]

B.

With these requirements in mind, we turn to the question of whether Ms. Johnson was stabilized before she was transferred to Crestview. *Cleland* guides us in analyzing

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[3]*Cleland* defined the term "appropriate medical screening" and in doing so inserted examples of improper motives. Notably, the term "appropriate medical screening" appears in subsection (a) of EMTALA not subsection (b), the subsection at issue in this case. However, *Cleland* specifically noted, after setting forth examples of improper motive, that "[s]imilarly, a discharge that to the knowledge of those conducting it left a patent with an 'emergency medical condition' in an 'unstable' condition would be actionable." *Cleland* at 272. Moreover, we see no rational reason to set forth differing standards when applying subsection (a) and (b).

[4]We, therefore, reject the approach of the Fourth Circuit taken in *In the Matter of Baby "K"*, 16 F.3d 590 (4th Cir. 1994). In *In the Matter of Baby "K"*, a hospital sought a declaratory judgment that it was not required under the EMTALA to provide respiratory support for a baby with anencephaly when presented to the hospital in respiratory distress. *Id.* at 592. The hospital contended that because aggressive treatment, such as respiratory support, would not serve any purpose, it was obligated only to provide the supportive care of nutrition, hydration, and warmth. *Id.* The hospital argued that the EMTALA required it only to provide the baby the same care it would provide to other babies suffering from anencephaly -- nutrition, hydration, and warmth. The court disagreed.

In the court's opinion, the statutory language of EMTALA is clear and it does not "allow the Hospital to fulfill its duty to provide stabilizing treatment by simply dispensing uniform treatment. Rather, the Hospital must provide that treatment necessary to prevent the material deterioration of each patient's emergency medical condition." *Id.* at 596. The court thus concluded that the hospital's uniform treatment of providing warmth, nutrition, and hydration was insufficient to avoid liability under EMTALA. Rather, the hospital was obligated to provide the treatment necessary to prevent the material deterioration of Baby K's condition which included respiratory support. *Id.*

a stabilization case. As with appropriate medical screening, the doctors are not charged with the duty under EMTALA of proper diagnosis; rather, they are charged with the duty of stabilizing a patient's condition as it as known to them. *Cleland*, 917 F.2d at 271. As noted by the District Court, there is a genuine issue of material fact as to this question. The day before Johnson was transferred, Johnson developed the symptoms of a urinary tract infection which were known to and treated by her physicians, but Johnson was transferred before doctors at Humana could determine whether the infection would respond to the antibiotic prescribed.

Drs. Richardson, Harbrecht, and Abou-Jaoude, the physicians involved in Johnson's care, attested that Johnson's condition was sufficiently stable to warrant discharge. Lynette Ferguson, Crestview's Director of Nursing, assessed Johnson's condition approximately two hours after her arrival. In her opinion, Johnson's condition was sufficiently stable to be treated at Crestview. Dr. Taiser Shatara, Johnson's admitting physician to Crestview, testified that, based on the information he had learned from the nurses at Humana, he believed that Johnson was an appropriate candidate for nursing home placement and stable when transferred. However, he testified that, had he known of the full history of Johnson's hospitalization, her suspected urinary tract infection, and elevated white count, he would not have accepted her as a patient at Crestview. Dr. John Stuy testified that, in his opinion, Johnson should have remained at Humana

until her doctors could review the results of the culture and determine whether the prescribed antibiotic would have successfully treated the infection.

In addition to this conflicting testimony, there is a dispute as to whether Johnson's condition was stabilized upon transfer but worsened by Crestview's insufficient treatment of Johnson. Most significantly, contrary to instructions given by Humana, Crestview staff did not suction Johnson's lungs nearly as often as recommended.

Humana has not appealed the District Court's holding that there is a genuine issue of material fact regarding whether Johnson was stabilized when she was discharged from Humana.

C.

Our analysis, however, does not cease with the conclusion that there exists a genuine of material fact regarding whether Johnson was stabilized when she was discharged from Humana. We must additionally determine, pursuant to *Cleland*, whether Humana acted with an improper motive when it discharged Johnson.

Joanna Nolte, a representative of Crestview Health Care Facility, testified that Nancy Fred explained to her "they had been trying to discharge and without any avail to other facilities and that she was getting a lot of pressure to discharge this person due to they knew they weren't going to get paid for this person." Nolte testified that Fred did not specifically mention who was placing pressure on her to

discharge Wanda, she said "she was just receiving pressure from the hospital that, you know, that we needed -- that she needed to find placement for this person." As acknowledged by the District Court, this statement suggests that some representative of the hospital may have been acting with an improper motive. However, as the District Court also concluded, there is no evidence that this improper motive caused Dr. Abou-Jaoude to issue Johnson's discharge order. Fred played no role in diagnosing Johnson as stabilized at the time of discharge. Even assuming financial considerations influenced Ms. Fred when pursuing a placement for Johnson, plaintiff has brought forth nothing more than speculation in support of her theory that the discharge papers were signed before Johnson was stabilized due to inappropriate economic considerations.

In support of its motion for summary judgment, defendant submitted the affidavits of Dr. David Richardson, Dr. Brian Harbrecht, and Dr. Abou-Jaoude. Dr. Richardson attested that he participated in Johnson's care and was certain that her financial condition was not a consideration in the medical decision to discharge her. Dr. Harbrecht attested that he consulted in and approved of Johnson's discharge and that her financial status played no role. Dr. Abou-Jaoude similarly attested that he was unaware of Johnson's financial status when he wrote the discharge order and that her financial status was not a factor in his decision to write the order. Moreover, Frank M. Miller, Chief of the Surgical Intensive

Care Unit and Chief of the Department of Surgery at Humana, testified that the attending physicians and residents "never have any idea what the financial condition of the patient is." In response, plaintiff brought forth no evidence suggesting that Johnson's treating physicians were aware of Johnson's financial condition. Thus, plaintiff failed to create a genuine issue of material fact as to causation. Without some evidence of a causal relation between the hospital employee possessing the improper motive and the discharge of the unstable patient, a plaintiff can not establish that a patient's discharge was caused by an improper motive. Accordingly, the District Court properly granted defendant summary judgment on the plaintiff's claim under the EMTALA.

IV.

In addition to the EMTALA claim, plaintiff brought a state law negligence claim against Humana for the acts of the surgical residents who treated Johnson. In order to hold the hospital liable for the negligent acts of the surgical residents, plaintiff argued they were ostensible agents of the hospital. The District Court held that the surgical residents at Humana were not ostensible agents and, therefore, granted summary judgment on behalf of Humana on the state law claim.

As noted by the District Court, Kentucky courts have relied heavily on the manner in which the principal holds itself out of the public in determining whether an ostensible agent or apparent authority relationship exits. In *Williams v.*

St. Claire Medical Center, 657 S.W.2d 590 (Ky. Ct. App. 1983), the court relied on the following definitions of ostensible agency:

> Apparent authority is not actual authority, but rather "is that which, by reason of prevailing usage or other circumstance, the agent is in effect held out by the principal as possessing. It is a matter of appearances, fairly chargeable to the principal and by which persons dealt with are deceived, and on which they rely."

Id. at 595 (quoting *Estell v. Barrickman*, 571 S.W.2d 650, 652 (Ky. Ct. App. 1978)).

> One who represents that another is his servant or other agent and thereby causes a third person justifiably to rely upon the care or skill of such apparent agent is subject to liability to third person for harm caused by the lack of care or skill of the one appearing to be a servant or other agent as if he were such.

Id. at 595-596 (quoting The Restatement (Second) of Agency § 267 (1958)). Because the hospital in *Williams* did nothing to inform the plaintiff that a nurse, an independent contractor whose actions were alleged to be negligent, was something

other than a hospital employee, the "hospital 'held-out' [the nurse] as an employee, thus creating an apparent agency." *Id.* at 596.

In *Paintsville Hospital Co. v. Rose*, 683 S.W.2d 255 (Ky. 1985), the Supreme Court of Kentucky specifically addressed the question of whether a hospital could be liable for the negligence of an emergency room physician who was not employed by the hospital. In a case where the hospital did nothing to alert the public that its emergency room physicians were not hospital employees, the court concluded that it is "unreasonable to put a duty on the patient to inquire of each person who treats him whether he is an employee or independent contractor of the hospital." *Id.* at 258 (citations omitted). The court further relied on the following statements from other courts on this issue in concluding that a hospital may be liable for the negligent acts of an independent emergency room physician:

> Absent notice to the contrary, therefore, plaintiff had the right to assume that the treatment received was being rendered through hospital employees and that any negligence associated with the treatment would render the hospital responsible. *Arthur v. St. Peters Hospital*, 169 N.J. Super. 575, 405 A.2d 443, 447 (1979).
>
> In our view, the critical question is whether

> the plaintiff, at the time of his admission to the hospital, was looking to the hospital for treatment of his physical ailments or merely viewed the hospital as the situs where his physician would treat him for his problems. *Grewe v. Mt. Clemens General Hospital*, 404 Mich. 240, 273 N.W.2d 429, 433 (1978).

Id. at 258.

Lastly, in *Floyd v. Humana of Virginia*, 787 S.W.2d 267 (Ky. Ct. App. 1989), the Kentucky Court of Appeals addressed the ostensible agency issue in a case where the patient had signed a form indicating that doctors in the hospital were independent contractors. In light of this evidence, the court concluded:

> we find the testimony of the appellant admitting that she had read and signed each of the admission forms to Humana of Virginia Hospital, Inc. d/b/a Humana Hospital University, which indicates her knowledge that the doctors were independent contractors and not agents of the hospital, to be determinative in this case. There was no representation or other action to induce appellant to believe that the physicians were

> employees or agents of Humana Hospital University; in addition, the admission form which appellant signed specially indicated same. Accordingly, there can be no valid argument that the ostensible agency doctrine would make Humana of Virginia liable in this case.

Id. at 270 (citing *Williams v. St. Claire Medical Center*, 657 S.W.2d 590 (Ky. Ct. App. 1983)).

In the case at hand, Humana's outpatient registration and authorization for medical treatment form states:

> INDEPENDENT STATUS OF
> PHYSICIANS, RESIDENTS AND
> MEDICAL STUDENTS.
>
> The medical treatment rendered to you during your hospital admission will be provided by physicians, residents and medical students (under the supervision of physicians and/or residents). These physicians, residents, and medical students are independent practitioners and are not employees or agents of the hospital.
>
> In addition, certain professional services rendered to you during your hospital

> admission will be provided by physicians, residents and medical students who are independent practitioners and are not employees or agents of Humana Hospital - University of Louisville. These services include but are not limited to the following: Radiology, Anesthesiology, Emergency Room, Pathology.
>
> The physicians providing treatment to you and the physicians providing the above mentioned professional services are not employees or agents of Humana Hospital - University of Louisville.

The record indicates that a form with this wording was specifically drafted for Johnson[5] on May 20, 1992 at 1:00 a.m., shortly after she arrived at the hospital. The form, however, was not signed by Johnson presumably because she was physically unable to do so. Thus, we are faced with a case where a hospital has overtly represented that its physicians are not its agents but where the patient has not seen or acknowledged the disclaimer. Whether or not Johnson read or signed the disclaimer is not dispositive. If it were dispositive, patients too critical to sign the consent form could sue residents for negligence whereas those able to read

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[5]Neither the parties nor the record indicate why an outpatient, rather than an inpatient, form was drafted for Johnson.

and sign the form could not. As stressed by Kentucky case law, the result should, instead, turn on whether the hospital holds its physicians out to be employees or something else. The passages cited to approvingly by the Kentucky courts focus particularly on the actions of the hospital. As noted by *Williams v. St. Claire Medical Center*, 657 S.W.2d 590 (Ky. Ct. App. 1983), "apparent authority is not actual authority, but rather 'is that which, by reason of prevailing usage or other circumstance, the agent is in effect held out by the principal as possessing. It is a matter of appearances, fairly chargeable to the principal and by which persons dealt with are deceived, and on which they rely.'" *Id.* at 595 (quoting *Estell v. Barrickman*, 571 S.W.2d 650, 652 (Ky. Ct. App. 1978)). And, as defined by the Restatement,

> One who represents that another is his servant or other agent and thereby causes a third person justifiably to rely upon the care or skill of such apparent agent is subject to liability to third person for harm caused by the lack of care or skill of the one appearing to be a servant or other agent as if he were such.

Id. at 595-596 (quoting The Restatement (Second) of Agency § 267 (1958)). Thus, under Kentucky law, the actions of the hospital, rather than the knowledge of the patient, is controlling in a case where the patient is unable to obtain

actual knowledge of the hospital's disclaimer. Here, Humana clearly attempted to alert the public that its physicians were not employees or agents of the hospital. The District Court, therefore, properly concluded that the surgical residents were not ostensible agents of the hospital. Accordingly, the dismissal of plaintiff's state law negligence is affirmed.

V.

For the foregoing reasons, the judgment of the District Court is **AFFIRMED**.

DAVID A. NELSON, Circuit Judge, concurring in part and dissenting in part. I agree with the court's disposition of Mrs. Johnson's negligence claim. As explained in Part IV of Judge Kennedy's opinion, the surgical residents could not properly be found to have been ostensible agents of defendant Humana - and absent an agency relationship, the residents' alleged negligence could not properly be imputed to Humana.

I disagree with the court's disposition of Mrs. Johnson's claim under the Emergency Medical Treatment and Active Labor Act. A covered hospital that is acting from an improper motive is prohibited by the Act from discharging a patient such as Mrs. Johnson for transfer to another facility without first having stabilized her medical condition. Stabilization has not been achieved if the transfer is likely to result in material deterioration of the patient's condition. Like my colleagues on the panel, I believe that there is a

genuine issue of fact as to whether Mrs. Johnson was stabilized at the time of her transfer from Humana to the Crestview facility. Unlike my colleagues, I believe that there is also a genuine issue of fact as to whether Humana acted from an improper motive. The issue is a material one, under our case law, so it seems to me that summary judgment ought not to have been granted on the statutory claim.

As far as the record discloses, to be sure, Dr. Abou-Jaoude -- the physician who issued Mrs. Johnson's discharge order -- was not knowingly influenced by improper financial considerations when he decided that it was appropriate for Mrs. Johnson to be discharged. But as Dr. Abou-Jaoude made clear in his deposition testimony, the discharge decision was not made in a vacuum. Mrs. Johnson was a "high-risk" patient who needed "skilled nursing care," Dr. Abou-Jaoude testified. He clearly would not have discharged her had he thought she was going to a boarding house, for example. The doctor talked to social worker Nancy Fred "multiple times" about Mrs. Johnson's discharge plan, and he engaged in "frequent discussions" with Ms. Fred about where Mrs. Johnson was going to go and who was going to take care of her. He specified placement in a skilled nursing care facility, according to his testimony, and he assumed that the social workers would locate a facility with nurses who could take care of the patient. As far as selection of a specific placement was concerned, Dr. Abou-Jaoude testified, "we just left it in the hands of the social workers. . . ."

The social workers' hands may not have been as clean as the doctor's apparently were. There is evidence from which a jury could infer that Ms. Fred was receiving pressure from Humana's administration to get Mrs. Johnson discharged, there being no prospect that Humana would ever be paid for taking care of this patient. There is evidence from which a jury could infer that the Crestview facility was not an appropriate placement for Mrs. Johnson. And given the extent to which Dr. Abou-Jaoude relied on Ms. Fred to locate a facility to which Mrs. Johnson could be transferred without any material deterioration in her condition, it seems to me that it would be permissible for a jury to find that rather than letting Mrs. Johnson stay put until an appropriate placement could be found elsewhere, Humana sent Mrs. Johnson to an inappropriate facility because Humana was not being paid. Such a finding would make Humana liable under the Act. Accordingly, I respectfully dissent from affirmance of the summary judgment for Humana on Mrs. Johnson's statutory claim.

UNITED STATES DISTRICT COURT
WESTERN DISTRICT OF KENTUCKY
AT LOUISVILLE

JANE ROBERTS, as Guardian for WANDA Y. JOHNSON — PLAINTIFF

v. — CIVIL ACTION NO. 3:93CV-544-S

GALEN OF VIRGINIA, INC., d/b/a UNIVERSITY OF LOUISVILLE HOSPITAL — DEFENDANT

MEMORANDUM OPINION

This matter is before the court on the motion of the defendant, Galen of Virginia, Inc., d/b/a University of Louisville Hospital (the "Hospital"), for partial summary judgment.

The complaint was filed in this court invoking our federal question jurisdiction. The plaintiff alleges (1) that the Hospital violated the Emergency Medical Treatment and Active Labor Act, 42 U.S.C. § 1395dd (§ 1395dd), (2) that the Hospital violated Wanda Y. Johnson's right to due process and equal protection of the law under the Constitutions of the United States and Kentucky, and (3) medical negligence on the part of the Hospital, its employees, and the physicians as its ostensible agents.

FACTS

Wanda Y. Johnson was injured May of 1992, when she was struck by a vehicle while walking. The parties agree that her injuries were life threatening and that she was critically ill. She was taken to the Hospital where she received treatment for her injuries over the course of the next two months. During her stay at the Hospital, she remained in a weakened state. She suffered a series of infections including pneumonia and urinary tract infections.

In July of 1992, the Hospital sought to transfer Johnson to a nursing home facility. Prior to her discharge, she demonstrated the symptoms of an active infection. She was placed on an antibiotic treatment and transferred on July 24, 1992, to Crestview, a licensed nursing facility. The day after arriving at Crestview, Johnson was taken from the nursing home and admitted to Midwest Medical Center, an Indianapolis hospital. She remained at that facility for several months.

Johnson has no medical insurance. Her hospital bills while at the Hospital surpassed the $300,000 mark.

I

A party moving for summary judgment has the burden of showing that there are no genuine issues of fact and that the movant is entitled to summary judgment as a matter of law. *Adickes v. S.H. Kress & Co.*, 398 U.S. 144, 151-60, 90 S. Ct. 1598, 16 L. Ed. 2d 142 (1970); *Felix v. Young*, 536 F.2d 1126, 1134 (6th Cir. 1976). Not every factual dispute

between the parties will prevent summary judgment. The disputed facts must be material. They must be facts which, under the substantive law governing the issue, might affect the outcome of the suit. *Anderson v. Liberty Lobby, Inc.*, 106 S.Ct. 2505, 2510 (1986). The dispute must also be genuine. The facts must be such that if they were proven at trial, a reasonable jury could return a verdict for the non-moving party. *Id.* at 2510. The disputed issue does not have to be resolved conclusively in favor of the non-moving party, but that party is required to present some significant probative evidence which makes it necessary to resolve the parties' differing versions of the dispute at trial. *First National Bank of Arizona v. Cities Service Co.*, 391 U.S. 253, 288-89 (1968). The evidence must be construed in a light most favorable to the party opposing the motion. *Bohn Aluminum & Brass Corp. v. Storm King Corp.*, 303 F.2d 425 (6th Cir. 1962).

The Hospital has moved for summary judgment, addressing Counts 1, 2, and 3 of the complaint. We will consider each of these arguments.

II

Count 1 of the complaint alleges that the defendant violated § 1395dd, when it transferred Johnson to Crestview.

Section 1395dd is designed to keep hospitals from "dumping" patients in need of emergency medical care. It provides that when any individual comes to the emergency department of a hospital and a request is made for a screening, an appropriate medical screening must be provided

to determine whether an emergency medical condition exists. If an emergency medical condition is found, the hospital must either provide stabilizing treatment or transfer the individual in accordance with the requirements of the statute. The plaintiff contends that the Hospital violated part (b) of § 1395dd in that it failed to stabilize her and transferred her without conforming with the requirements of part (c). The Hospital argues that it provided stabilizing treatment, that Johnson was stable when she was transferred to Crestview, and that the plaintiff has failed to prove that the Hospital acted with an improper motive. Therefore, the Hospital contends, summary judgment is warranted on this count.

The plaintiff does not allege that the screening given by the Hospital in May, 1992 was inappropriate. Furthermore, it is undisputed that at the time Johnson arrived at the Hospital she presented an emergency medical condition. The parties disagree on the issues of whether Johnson was stabilized when she was transferred to Crestview and whether the Hospital acted with an improper motive. We will address both issues.

A

If Johnson was stable when she was transferred, then the Hospital did not violate § 1385dd(b). This is a material fact.

The Hospital presents the affidavits of the treating physicians and those involved in Johnson's discharge. They are in agreement that Johnson was stable when she was

transferred to Crestview.

Plaintiff presents the testimony of Dr. Stuy, the physician who treated Johnson at Midwest Medical Center in Indianapolis. Portions of Dr. Stuy's deposition testimony follow:

> Q. All right. Doctor we're -- we're talking about what is expected or what is reasonable medical care in a situation where a doctor has identified what appears to be active infection and has prescribed antibiotic treatment for that infection. Is there a certain period of time that you normally would expect to have to monitor a patient to determine whether the antibiotics that you have prescribed are going to adequately treat the infection that you found?
>
> A. Well, what you would typically do is obtain a culture of the potential source of infection. So in this case, you would obtain a sample of urine and send it to the laboratory for culture and sensitivity; by that, the laboratory would try to grow bacteria to see if there is an active infection, identify those bacteria and then they can in the laboratory check various antibiotics and see if those antibiotics would successfully kill or eliminate

> those bacteria. So you identify the organism then you can actually check a number of antibiotics and see if those are successful in treating that particular infection.
>
> Q. Generally is this something that takes a period of days?
>
> A. Within 48 hours they should be able to render a preliminary report and within 72 hours they should be able to give a positive identification
>
> Q. If a physician does not follow through with this analysis of the antibiotic treatment that he's administered, would that generally in your opinion be outside the normal standard of good medical care?
>
> A. . . . I think in this particular patient because she is immunocompromised, she's lacking a spleen, also the fact that she has a predisposition to pneumonia, she has a predisposition to urinary tract infection because of a catheter residing in her bladder and the fact that she's been in the hospital a long time and, therefore, exposed to bacteria

and organisms that are not typically more easily treated, I think she should have had a culture obtained and looked at prior to her release to be sure that antibiotic would address that infection successfully.

Q. Dr. Stuy, based on your review of Wanda Johnson's medical records from the University of Louisville during the last couple of days that she was there, was her condition such that -- that she required continuous medial attention in a hospital setting?

A. I think, as I mentioned previously, that cultures of the presumed infection should have been obtained and looked at first prior to transfer to be sure that the antibiotics that they picked would successfully treat that infection. If they picked an antibiotic that would not successfully treat an infection and she were released, because of her generally immunocompromised state, lack of spleen, prolonged hospital stay, foreign bodies in the form of catheters, there is some risk that the infection would get worse and could cause her harm.

(Text omitted)

Q. All right. Was Wanda Johnson's medical condition such that if she did not have available to her medical attention her condition could possibly have deteriorated very quickly?

A. Yes.

Q. Describe again for us the types of health risks or conditions that under Wanda Johnson's condition she may have suffered from, not necessarily that she did suffer from, but the risks that she -- that were involved in her condition.

A. Her ability to fight off infection was compromised. If the infection were not to be checked or treated with the antibiotics successfully, it could spread into the bloodstream and cause a condition called sepsis which if not treated quickly can result in death.

Q. Doctor, based on your experience and treatment of Wanda Johnson and your

review of her medical records at Humana Hospital, University of Louisville, do you have an opinion as to whether Wanda Johnson's transfer from Louisville to Indianapolis on July 24, 1992 was consistent with accepted medical standard of care?

A. I don't think it was.

Q. Doctor, would the risks that you've previously told us about, would a -- trip in an ambulance from Louisville to Indianapolis, that period of time that it would take to -- to travel that distance, would you consider that a dangerous situation for Wanda Johnson in her condition?

A. I think she should have remained in the hospital setting until they could document that the antibiotics that they picked could successfully treat the suspected infection.

Q. Doctor, could you tell us a little bit about what injuries, if any, or what deterioration in Wanda Johnson's condition you noticed as having been caused by her transfer from the University of Louisville

Hospital?

A. When I met her Saturday morning, she had a very high fever, she was breathing very rapidly, clinically she demonstrated evidence of pneumonia and by x-ray she also demonstrated evidence of pneumonia. Her white blood cell count was high. She was flushed, sweaty, hot. I felt she demonstrated all the physical findings and laboratory findings consistent with early sepsis or overwhelming infection.

Q. And --

A. If this is not treated, then it results in death.

Q. Doctor, was Wanda Johnson's condition substantially worse when you saw her on Saturday morning than what it appeared to have been just prior to her transfer from the University of Louisville Hospital?

A. Her res -- yes, based on the notes that I read, her -- her respiratory rate was very rapid, her pulse was very rapid, her pulse was

very rapid, her temperature was 105 at the nursing home just prior to transfer to the hospital which is higher than at the hospital -- Humana Hospital, so yes, she was worse than -- than at Humana.

A key issue in this case is whether Johnson was "stabilized." "The term 'stabilized,' means that 'no material deterioration of the condition is likely, within reasonable medical probability, to result from the transfer of the individual from a [hospital]' 42 U.S.C. Section 1395dd(e)(4)(B)." *Thornton v. Southwest Detroit Hosp.*, 895 F.2d 1131, 1134 (6th Cir. 1990). Dr. Stuy's testimony does not mirror the language of the statute. However, it can be read to express the belief that Johnson was not stable, due to her apparent infection, immunocompromised state, and prior history of infections. Furthermore, there is evidence that Dr. Stuy's opinion was not based solely on the outcome. Dr. Stuy testified that considering Johnson's immunocompromised condition, the signs of infection, and her medical history she should not have been transferred when she was. Therefore, the plaintiff's contention that she was not stable is based on facts existing at the time of her discharge, not "only on the outcome."

Viewing the evidence in the light most favorable to the non-moving party, we find that there is a genuine issue of a material fact with regard to the issue of stabilization.

B

The Hospital also contends that it did not act with an improper motive. The Sixth Circuit recognizes the presence of an improper motive as an essential element in a claim for relief under § 1395dd. *Cleland v. Bronson Health Care Group, Inc.*, 917 F.2d 266, 271 (6th Cir. 1990). This requirement applies to stabilization with even greater force. *Id.* A hospital violates the statute where it refuses treatment or gives cursory treatment to a person for improper reasons. *Id.* at 272. Without proof of an improper motive, failure to stabilize is not actionable under the statute. *Cleland*, 917 F.2d 266; *Hines*, 827 F. Supp. 426.

The plaintiff offers the testimony of Joanna Nolte, the Crestview patient recruiter. Nolte spoke with Nancy Fred, a social worker with the Hospital, concerning placement of Johnson in a nursing home. An excerpt from Nolte's deposition testimony follows:

> Q. Okay. Tell us how you first learned of a patient named Wanda Johnson.
>
> A. The social worker that was handling her case asked me if I would be willing to take a look at a patient that they had been trying to discharge and without any avail to other facilities and that she was getting a lot of pressure to discharge this person due to they knew they were not going to get paid for

this person.

Q. Do you recognize the name Nancy Fred?

A. Correct, it was Nancy Fred.

This statement is admissible under Rule 801(d)(2)(D) as non-hearsay, since it is a statement offered against a party and was allegedly made "by the party's agent or servant concerning a matter within the scope of the agency or employment, made during the existence of the relationship. . . ." There is a difference of opinion concerning whether this statement was ever made. However, viewing this evidence in the light most favorable to the plaintiff, it does tend to establish a genuine issue of fact concerning whether the Hospital acted with an improper motive.

The Hospital argues that the decision to transfer Johnson was a medical choice and was not influenced by Johnson's apparent indigence. This argument presents an issue as to causation, which is not presently before the court. The Hospital's argument essentially is that regardless of the administration's motive, the doctors themselves made an independent medical decision to transfer the patient. Therefore, they contend that, since the transfer was not effected due to an improper motive, the Hospital cannot be held liable under the statute. *See, e.g. Hines*, 827 F. Supp.

426. Since the causation issue is not squarely before us, we must confine our ruling to those issues directly in contention.

The plaintiff has established that there is a genuine question of fact with regard to the issue of improper motive on the part of the Hospital. Therefore, the Hospital's motion for summary judgment will be denied with regard to this count.

III

Count 2 of the complaint alleges that the Hospital acted under color of state law when it transferred Johnson to Crestview. Therefore, the plaintiff contends, the Hospital was required to comply with the constitutional requirements of equal protection and due process.

In order to proceed with these claims, the plaintiff must satisfy the state action requirement of the Fourteenth Amendment. *Blum v. Yaretsky*, 457 U.S. 991 (1982). "That amendment erects no shield against merely private conduct, however discriminatory or wrongful." *Id.* at 1002.

The Supreme Court has set forth the following three tests to determine whether challenged conduct amounts to state action: (1) the public function test; (2) the state compulsion test; and (3) the symbiotic relationship or nexus test. *Wolotsky v. Huhn*, 960 F.2d 1331, 1335 (6th Cir. 1992) (citations omitted). The plaintiff argues that under either the public function or nexus test, the Hospital's conduct was state action.

The plaintiff first attempts to establish that the

Hospital is fulfilling a public function. The test requires that "the private entity exercise powers which are traditionally exclusively reserved to the state . . . ." *Wolotsky v. Huhn*, 960 F.2d 1331, 1335 (6th Cir. 1992). However, the provision of hospital services is not a power which has traditionally been exclusively reserved to the state. Moreover, the plaintiff has not established that in the absence of the Hospital, the state is required to provide these hospital services. *See, Blum*, 457 U.S. at 1011. Therefore, the Hospital is not fulfilling a public function.

Next, the plaintiff asserts that "there is a sufficiently close nexus between the State and the challenged action of the regulated entity so that the action of the latter may be fairly treated as that of the State itself." Plaintiff's Memorandum at 30 (citing *Jackson v. Metropolitan Edison Co.*, 419 U.S. 345, 351 (1974)). The plaintiff contends that since (1) the hospital was previously operated by the University, (2) the University expressed certain desires in its agreements with the Hospital, (3) the Hospital received appropriations from the state, and (4) the Hospital participated in Medicaid, there was a sufficient nexus between the state and the Hospital to amount to state action.

The ultimate issue is whether the alleged infringement of rights is "fairly attributable to the state." *Crowder v. Conlan*, 740 F.2d 447, 449 (6th Cir. 1984). In *Crowder*, a physician sued a private hospital for civil rights violations alleged to have occurred in the restriction of his staff

privileges. *Id.* The *Crowder* court found no state action in the hospital's decision where the hospital received considerable revenues from the government, the hospital was subject to extensive regulation, and the hospital's physical plant was owned by the county and leased to the defendant. The court noted that the county was not involved in the day-to-day operations of the hospital nor was it involved in the challenged action. *Id.*

In *Blum*, the issue before the Court was whether state action attaches to a nursing facility's summary discharge or transfer of a patient. *Blum*, 457 U.S. 991. The nursing homes in question were receiving federal funds for at least 90% of their patients. *Id.* Although the plaintiff in *Blum* actually sued state officials for the conduct of private parties, the reasoning applies in the instant case as well. *Crowder*, 740 F.2d at 450. The Court in *Blum* noted, "[t]hat programs undertaken by the State result in substantial funding of the activities of a private entity is no more persuasive than the fact of regulation of such an entity in demonstrating that the State is responsible for decisions made by the entity in the course of business." *Blum*, 457 U.S. at 1011 (holding that neither substantial regulation nor funding of a private enterprise amounted to state action). The Court found that the plaintiffs had failed to establish state action, because the decisions were independently made by private individuals. *Id.* at 1008. The state had not exercised such coercion or given encouragement that the choice must be deemed that of

the state. *Id.* at 1004.

The United States Supreme Court and the Sixth Circuit Court of Appeals have found state action lacking in similar circumstances. We find state action lacking here as well. As in *Blum* and *Crowder* the decision in this case to transfer Johnson was not that of the state but that of the privately owned and operated hospital.

Therefore, the defendant's motion with respect to this count will be granted.

IV

Count 3 of the complaint alleges negligence on the part of the Hospital through its corporate acts, the acts of its employees, and the acts of the doctors as its "ostensible agents."

The Hospital challenges the allegations in this count on the ground that the medical residents were not the ostensible agents of the Hospital. The uncontroverted evidence is that the Hospital required conscious patients to sign a form acknowledging the independent status of physicians. The Hospital asserts that it did not hold itself out as ostensible employer of the medical residents and should not be liable for their alleged negligence.

It is apparent from a review of the Kentucky cases concerning ostensible agency that the manner in which the principal (the Hospital) holds itself out to the public is controlling. In *Williams v. St. Clare Medical Center*, 657 S.W.2d 590 (Ky. App. 1983), the court of appeals adopted

ostensible agency to hold a hospital liable for the negligence of an independent contractor nurse. The *Williams* court noted:

> Apparent authority is not actual authority, but rather "is that which, by reason of prevailing usage or other circumstance, the agent is in effect held out by the principal as possessing. It is a matter of appearance, fairly chargeable to the principal and by which persons dealt with are deceived, and on which they rely.

Id. at 595 (quoting *Estell v. Barrickman*, 571 S.W.2d 650, 652 (Ky. App. 1978). By taking no action to give notice to the contrary, the defendant in *Williams* held out the nurse as its employee and was held liable for his negligence.

This theory was applied to emergency room doctors by the Supreme Court of Kentucky in *Paintsville Hosp. Co. v. Rose*, 683 S.W.2d 255 (Ky. 1985). In that case, the court held that a "general representation to the public [was] implied from the circumstances." *Id.* at 256. Although proof of reliance on the part of the unconscious patient was lacking, the court found the emergency room doctor to be the ostensible agent of the hospital, focussing on the appearance, the representations, and the manifestations of the principal and its apparent agent. *Id.* at 257.

The Kentucky courts recognize that the determining factor in a question of ostensible agency is whether the

principal holds out the third party as its agent. The undisputed evidence in this case is that the Hospital did not hold out these doctors as its agents. In fact, it required each conscious patient to sign an admission form acknowledging that the doctors were in fact independent contractors.[1] According to the *Rose* court, where the hospital holds out the doctors as its agents, the plaintiff need not prove actual reliance to establish apparent agency. Conversely, when the evidence demonstrates that the hospital as a matter of routine practice disclaims an agency relationship with its resident doctors, apparent agency is not shown even if the patient is unconscious and does not specifically receive notice of the disclaimer. Just as reliance is not necessary to prove apparent agency if the hospital does not routinely disclaim it, reliance is not necessary to disprove apparent agency where the hospital's routine is to disclaim it.

The test is whether the Hospital held out these doctors as its agents. The undisputed evidence is that it did not. Therefore, the defendant's motion for summary judgment with regard to this portion of Count 3 will be granted.

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[1]*See, e.g., Floyd v. Humana of Virginia, Inc.*, 787 S.W.2d 267, 270 (Ky. App. 1989), *discr. rev. denied* (holding that where the plaintiff had signed a form acknowledging the independent status of the doctors and there was no representation or other action to induce one to believe that the doctors were the agents of the hospital, there can be no valid argument that the ostensible agency doctrine should apply.)

CONCLUSION

The Hospital has moved for partial summary judgment. As to Count 1 of the complaint, there are material issues of fact with regard to the issues as presented to the court. Therefore, the Hospital's motion will be denied for that count. However, the plaintiff did not establish state action, and accordingly the motion will be granted as to Count 2. As to Count 3, the doctrine of ostensible agency does not apply to the Hospital in relation to the independent contractor doctors involved in treating Johnson at the Hospital. Therefore, the Hospital's motion will be granted with regard to that portion of Count 3 which asserts a claim under the theory of ostensible agency.

An order in accordance with this opinion will be entered this date.

This 13th day of November, 1995.

/s/Charles R. Simpson III
Charles R. Simpson III
Chief Judge
United States District Court

cc: Counsel of Record

UNITED STATES DISTRICT COURT
WESTERN DISTRICT OF KENTUCKY
AT LOUISVILLE

JANE ROBERTS, as Guardian for WANDA Y. JOHNSON — PLAINTIFF

v. — CIVIL ACTION NO. 3:93CV-544-S

GALEN OF VIRGINIA, INC., d/b/a UNIVERSITY OF LOUISVILLE HOSPITAL — DEFENDANT

ORDER

This matter having come before the court on the motion of the defendant for partial summary judgment, and the court having considered said motion and being otherwise sufficiently advised, **IT IS HEREBY ORDERED**:

1. That the defendant's motion with respect to count 1 of the complaint is **DENIED**.

2. That the defendant's motion with respect to count 2 of the complaint is **GRANTED** and that count is **DISMISSED WITH PREJUDICE**.

3. That the defendant's motion with respect to that portion of count 3 as asserts a claim against the defendant for the alleged negligence of the independent contractor

physicians under the doctrine of ostensible agency is **GRANTED**, and that portion of count 3 is **DISMISSED WITH PREJUDICE**.

4. Inasmuch as count 1 of the complaint has not been dismissed, this court retains federal question jurisdiction, and the defendant's motion to remand is **DENIED**.

IT IS SO ORDERED this 13th day of November, 1995.

/s/ Charles R. Simpson III
Charles R. Simpson III,
Chief Judge
United States District Court

cc: Counsel of Record

UNITED STATES DISTRICT COURT
WESTERN DISTRICT OF KENTUCKY
AT LOUISVILLE

JANE ROBERTS, as Guardian for WANDA Y. JOHNSON — PLAINTIFF

v. — CIVIL ACTION NO. 3:93CV-544-S

GALEN OF VIRGINIA, INC., d/b/a UNIVERSITY OF LOUISVILLE HOSPITAL — DEFENDANT

MEMORANDUM OF OPINION

This matter is before the court on the motion of the defendant for reconsideration of the court's order dated November 13, 1995. In that order we denied the defendant's motion for summary judgment with respect to count 1 of the complaint and further denied the defendant's motion for remand.

We discussed the relevant facts in our previous memorandum opinion and see no need to repeat them here.

I

In its previous memoranda, the defendant argued that since it did not act with an improper motive, the plaintiff's claim must fail. We did not consider the defendant's

argument concerning causation, as it was not squarely before us. However, the defendant's motion for reconsideration raises this argument and we will now address it.

A hospital may be liable under the Emergency Medical Treatment and Active Labor Act (EMTALA) for "failing to stabilize" a patient's condition prior to transferring the patient. *Cleland v. Bronson Health Care Group, Inc.*, 917 F.2d 266, 271 (6th Cir. 1990) (quoting *DeBerry v. Sherman Hosp. Assoc.*, 741 F. Supp. 1302 (N.D. III. 1990). However, the plaintiff must establish that discriminatory motive caused the failure to stabilize. *Cleland*, 917 F.2d at 271. EMTALA provides protection not from medical malpractice but from discriminatory treatment or transfer. *Id.* at 272 (noting that EMTALA does not apply to hospital which acts in the same manner as it would for the usual paying patient).

We determined in our prior opinion in this matter that there was an issue of fact with regard to whether Johnson was stable when she was transferred. We also noted that the plaintiff presented some evidence as to an improper motive on the part of the defendant hospital. However, we expressly confined our decision to those issues and did not address the issue of causation.

The defendant contends that the decision to transfer Wanda Y. Johnson was based entirely on her medical condition, as determined by her physicians who were unaware of her financial status. It presents the affidavits of the doctors responsible for Johnson's discharge. Defendant's

Memorandum, Ex. B (affidavits of Drs. Richardson, Harbrecht, and Abou-Jaoude). Each of these doctors states that Johnson was stable when she was transferred. They also state that her financial condition did not affect their medical decision to authorize her transfer.

If "[i]n the hospital's opinion, the patient was stable, and they would have believed that a patient with any differing characteristics would have been stable[,] "then" within the meaning of the Act [EMTALA] the hospital did its duty to stabilize" Johnson. *Cleland*, 917 F.2d at 271. This is true even if it turns out the "patient was in fact not stabilized[.]" *Hines*, 827 F. Supp. at 431 (explaining *Cleland*, which held that hospital fulfilled its obligation although the patient was later determined not to have been stable).

The defendant has the burden of showing that there were no genuine issues of fact and that it is entitled to summary judgment as a matter of law. *Adickes v S.H. Kress & Co.*, 398 U.S. 144, 151-60 (1970); *Felix v. Young*, 536 F.2d 1126, 1134 (6th Cir. 1976). In this case the defendant presented evidence tending to negate the causative effect of any improper motive held by the defendant. Thus, the plaintiff was required to present some significant probative evidence making it necessary to resolve the parties' differing versions of the dispute at trial. *First National Bank of Arizona v. Cities Service Co.*, 391 U.S. 253, 288-89 (1968). Yet, the plaintiff presented no evidence that either the medical opinion that Johnson was stable or the decision to authorize

her transfer was caused by an improper motive. There is no issue as to this material fact and summary judgment is warranted.

For the foregoing reasons, the defendant's motion for reconsideration will be granted. The EMTALA claim in count 1 of the complaint will be dismissed.

The defendant, in its original motion for summary judgment, asked the court to remand. Remand in this case is improper, inasmuch as the suit was originally filed with this court. Yet, the plaintiff filed this suit invoking our federal question jurisdiction, and all that remains of the plaintiffs complaint is a claim for medical negligence under state law. "Needless decision of state law should be avoided both as a matter of comity and to promote justice between the parties, by procuring for them a surer-footed reading of applicable law." *United Mine Workers of America v. Gibbs*, 383 U.S. 715, 726 (1966). The *Gibbs* court noted that where state claims are before the court on pendent jurisdiction, "if the federal claims are dismissed before trial, even though not insubstantial in a jurisdictional sense, the state claims should be dismissed as well." *Id.*

For the above reasons, the federal EMTALA claim in count 1 of the complaint will be dismissed with prejudice and the pendent state law claim in what remains of count 3 will be dismissed for lack of jurisdiction in this court.

An order in accordance with this opinion will be entered this date.

This 15th day of February, 1996.

/s/ Charles R. Simpson III
Charles R. Simpson III
Chief Judge
United States District Court

cc: Counsel of Record

UNITED STATES DISTRICT COURT
WESTERN DISTRICT OF KENTUCKY
AT LOUISVILLE

JANE ROBERTS, as Guardian for WANDA Y. JOHNSON — PLAINTIFF

v. — CIVIL ACTION NO. 3:93CV-544-S

GALEN OF VIRGINIA, INC., d/b/a UNIVERSITY OF LOUISVILLE HOSPITAL — DEFENDANT

ORDER

This matter having come before the court on the motion of the defendant for reconsideration of the court's order dated November 13, 1995, and the court having considered said motion and being otherwise sufficiently, **IT IS HEREBY ORDERED:**.

1. The motion is **GRANTED** and the federal EMTALA claim in count 1 of the complaint is **DISMISSED WITH PREJUDICE**.

2. The sole remaining claim under what remains of count 3 of the complaint asserts a state law cause of action. That count is **DISMISSED WITHOUT PREJUDICE. FEDERAL QUESTION JURISDICTION IS LACKING.**

IT IS SO ORDERED.

This 15th day of February, 1996.

/s/ Charles R. Simpson III
Charles R. Simpson III
Chief Judge
United States District Court

cc: Counsel of Record

42 U.S.C. § 1395dd. Examination and treatment for emergency medical conditions and women in labor

(a) Medical screening requirement. In the case of a hospital that has a hospital emergency department, if any individual (whether or not eligible for benefits under this title [42 USCS §§ 1395 et seq.]) comes to the emergency department and a request is made on the individual's behalf for examination or treatment for a medical condition, the hospital must provide for an appropriate medical screening examination within the capability of the hospital's emergency department, including ancillary services routinely available to the emergency department, to determine whether or not an emergency medical condition (within the meaning of subsection (e)(1)) exists.

(b) Necessary stabilizing treatment for emergency medical conditions and labor. (1) In general. If any individual (whether or not eligible for benefits under this title [42 USCS §§ 1395 et seq.]) comes to a hospital and the hospital determines that the individual has an emergency medical condition, the hospital must provide either -

(A) within the staff and facilities available at the hospital, for such further medical examination and such treatment as may be required to stabilize the medical condition, or

(B) for transfer of the individual to another medical facility in accordance with subsection

(c).

(2) Refusal to consent to treatment. A hospital is deemed to meet the requirement of paragraph (1)(A) with respect to an individual if the hospital offers the individual the further medical examination and treatment described in that paragraph and informs the individual (or a person acting on the individual's behalf) of the risks and benefits to the individual of such examination and treatment, but the individual (or a person acting on the individual's behalf) refuses to consent to the examination and treatment. The hospital shall take all reasonable steps to secure the individual's (or person's) written informed consent to refuse such examination and treatment.

(3) Refusal to consent to transfer. A hospital is deemed to meet the requirement of paragraph (1) with respect to an individual if the hospital offers to transfer the individual to another medical facility in accordance with subsection © and informs the individual (or a person acting on the individual's behalf) of the risks and benefits to the individual of such transfer, but the individual (or a person acting on the individual's behalf) refuses to consent to the transfer. The hospital shall take all reasonable steps to secure the individual's (or person's) written informed consent to refuse such transfer.

© **Restricting transfers until individual stabilized.** (1) Rule. If an individual at a hospital has an emergency medical condition which has not been stabilized (within the meaning

of subsection (e)(3)(B)), the hospital may not transfer the individual unless -

(A)(I) the individual (or a legally responsible person acting on the individual's behalf) after being informed of the hospital's obligations under this section and of the risk of transfer, in writing requests transfer to another medical facility,

(ii) a physician (within the meaning of section 1861(r)(1) [42 USCS § 1395x(r)(1)]) has signed a certification that based upon the information available at the time of transfer, the medical benefits reasonably expected from the provision of appropriate medical treatment at another medical facility outweigh the increased risks to the individual and, in the case of labor, to the unborn child from effecting the transfer, or

(iii) if a physician is not physically present in the emergency department at the time an individual is transferred, a qualified medical person (as defined by the Secretary in regulations) has signed a certification described in clause (ii) after a physician (as defined in section 1861(r)(1) [42USCS § 1395x(r)(1)]), in consultation with the person, has made the determination described in such

clause, and subsequently countersigns the certification; and

(B) the transfer is an appropriate transfer (within the meaning of paragraph (2)) to that facility.

A certification described in clause (ii) or (iii) of subparagraph (A) shall include a summary of the risks and benefits upon which the certification is based.

(2) Appropriate transfer. An appropriate transfer to a medical facility is a transfer -

(A) in which the transferring hospital provides the medical treatment within its capacity which minimizes the risks to the individual's health and, in the case of a woman in labor, the health of the unborn child;

(B) in which the receiving facility -

(I) has available space and qualified personnel for the treatment of the individual, and

(ii) has agreed to accept transfer of the individual and to provide appropriate medical treatment;

© in which the transferring hospital sends to the receiving facility with all medical records (or copies thereof), related to the emergency condition for which the individual has presented, available at the time of the transfer, including records related to the individual's

emergency medical condition, observations of signs or symptoms, preliminary diagnosis, treatment provided, results of any tests and the informed written consent or certification (or copy thereof) provided under paragraph (1)(A), and the name and address of any on-call physician (described in subsection (d)(1)(C)) who has refused or failed to appear within a reasonable time to provide necessary stabilizing treatment;

(D) in which the transfer is effected through qualified personnel and transportation equipment, as required including the use of necessary and medically appropriate life support measures during the transfer; and

(E) which meets such other requirements as the Secretary may find necessary in the interest of the health and safety of individuals transferred.

(d) Enforcement. (1) Civil monetary penalties. (A) A participating hospital that negligently violates a requirement of this section is subject to a civil money penalty of not more than $50,000 (or not more than $25,000 in the case of a hospital with less than 100 beds) for each such violation. The provisions of section 1128A [42 USCS § 1320a-7a] (other than subsections (a) and (b)) shall apply to a civil money penalty under this subparagraph in the same manner as such

provisions apply with respect to a penalty or proceeding under section 1128A(a) [42 USCS § 1320a-7a(a)].

(B) Subject to subparagraph (C), any physician who is responsible for the examination, treatment, or transfer of an individual in a participating hospital, including a physician on-call for the care of such an individual, and who negligently violates a requirement of this section, including a physician who -

> (I) signs a certification under subsection (c)(1)(A) that the medical benefits reasonably to be expected from a transfer to another facility outweigh the risks associated with the transfer, if the physician knew or should have known that the benefits did not outweigh the risks, or
>
> (ii) misrepresents an individual's condition or other information, including a hospital's obligations under this section,

is subject to a civil money penalty of not more than $50,000 for each such violation and, if the violation is gross and flagrant or is repeated, to exclusion from participation in this title [42 USCS §§ 1395 et seq.] and State health care programs. The provisions of section 1128A [42 USCS § 1320a-7a] (other than the first and second sentences of subsection (a) and subsection (b)) shall apply to a civil money penalty and exclusion under this subparagraph in the same

manner as such provisions apply with respect to a penalty, exclusion, or proceeding under section 1128A(a) [42 USCS § 1320a-7a(a)].

© If, after an initial examination, a physician determines that the individual requires the services of a physician listed by the hospital on its list of on-call physicians (required to be maintained under section 1866(a)(1)(I) [42 USCS § 1395cc(a)(1)(I)]) and notifies the on-call physician and the on-call physician fails or refuses to appear within a reasonable period of time, and the physician orders the transfer of the individual because the physician determines that without the services of the on-call physician the benefits of transfer outweigh the risks of transfer, the physician authorizing the transfer shall not be subject to a penalty under subparagraph (B). However, the previous sentence shall not apply to the hospital or to the on-call physician who failed or refused to appear.

(2) Civil enforcement. (A) Personal harm. Any individual who suffers personal harm as a direct result of a participating hospital's violation of a requirement of this section may, in a civil action against the participating hospital, obtain those damages available for personal injury under the law of the State in which the hospital is located, and such equitable relief as is appropriate.

(B) Financial loss to other medical facility. Any medical facility that suffers a financial loss as a direct result of a participating hospital's violation of a requirement of this

section may, in a civil action against the participating hospital, obtain those damages available for financial loss, under the law of the State in which the hospital is located, and such equitable relief as is appropriate.

© Limitations on actions. No action may be brought under this paragraph more than two years after the date of the violation with respect to which the action is brought.

(3) Consultation with peer review organizations. In considering allegations of violations of the requirements of this section in imposing sanctions under paragraph (1), the Secretary shall request the appropriate utilization and quality control peer review organization (with a contract under part B of title XI [42 USCS §§ 1320c et seq.]) to assess whether the individual involved had an emergency medical condition which had not been stabilized, and provide a report on its findings. Except in the case in which a delay would jeopardize the health or safety of individuals, the Secretary shall request such a review before effecting a sanction under paragraph (1) and shall provide a period of at least 60 days for such review.

(e) Definitions. In this section:

(1) The term "emergency medical condition" means -

(A) a medical condition manifesting itself by acute symptoms of sufficient severity (including severe pain) such that the absence of immediate medical attention could reasonably be expected to result in -

(I) placing the health of the

individual (or, with respect to a pregnant woman, the health of the woman or her unborn child) in serious jeopardy,

(ii) serious impairment to bodily functions, or

(iii) serious dysfunction of any bodily organ or part; or

(B) with respect to a pregnant women who is having contractions -

(I) that there is inadequate time to effect a safe transfer to another hospital before delivery, or

(ii) that transfer may pose a threat to the health or safety of the woman or the unborn child.

(2) The term "participating hospital" means hospital that has entered into a provider agreement under section 1866 [42 USCS § 1395cc].

(3)(A) The term "to stabilize" means, with respect to an emergency medical condition described in paragraph (1)(A), to provide such medical treatment of the condition as may be necessary to assure, within reasonable medical probability, that no material deterioration of the condition is likely to result or occur during the transfer of the individual from a

facility, or, with respect to an emergency medical condition described in paragraph (1)(B), to deliver (including the placenta).

(B) The term "stabilized" means, with respect to an emergency medical condition described in paragraph (1)(A), that no material deterioration of the condition is likely, within reasonable medical probability, to result from or occur during the transfer of the individual from a facility, or, with respect to an emergency medical condition described in paragraph (1)(B), that the woman has delivered (including the placenta).

(4) The term "transfer" means the movement (including the discharge) of an individual outside a hospital's facilities at the direction of any person employed by (or affiliated or associated, directly or indirectly, with) the hospital, but does not include such a movement of an individual who (A) has been declared dead, or (B) leaves the facility without the permission of any such person.

(5) The term "hospital" includes a rural primary care hospital (as defined in section 1861(mm)(1) [42 USCS § 1395x(mm)(1)]).

(f) Preemption. The provisions of this section do not preempt any State or local law requirement, except to the extent that the requirement directly conflicts with a requirement of this section.

(g) Nondiscrimination. A participating hospital that has specialized capabilities or facilities (such as burn units, shock-trauma units, neonatal intensive care units, or (with respect to

rural areas) regional referral centers as identified by the Secretary in regulation) shall not refuse to accept an appropriate transfer of an individual who requires such specialized capabilities or facilities if the hospital has the capacity to treat the individual.

(h) No delay in examination or treatment. A participating hospital may not delay provision of an appropriate medical screening examination required under subsection (a) or further medical examination or treatment required under subsection (b) in order to inquire about the individual's method of payment or insurance status.

(I) Whistleblower protections. A participating hospital may not penalize or take adverse action against a qualified medical person described in subsection (c)(1)(A)(iii) or a physician because the person or physician refuses to authorize the transfer of an individual with an emergency medical condition that has not been stabilized or against any hospital employee because the employee reports a violation of this section.

(Aug. 14, 1935, ch 531, Title XVIII, Part C, § 1867, as added April 7, 1986, P. L. 99-272, title IX, Subtitle A, Part 1, Subpart B, § 9121(b), 100 Stat. 164; Oct. 21, 1986, P. L. 99-514, Title XVIII, Subtitle C, Ch 1, § 1895(b)(4), 100 Stat. 2933; Dec. 22, 1987, P. L. 100-203, Title IV, Subtitle A, Part 1, § 4009(a)(1), (2), 101 Stat. 1330-56; July 1, 1988, P. L. 100-360, Title IV, Subtitle B, § 411(b)(8)(A)(I), 102 Stat. 772); Oct. 13, 1988, P. L. 100-485, title VI, § 608(d)(18)(E), 102 Stat. 2419; Dec. 19, 1989, P. L. 101-239, Title VI,

Subtitle A, Part 1, Subpart A, § 6003(g)(3)(D)(xiv), Part 3, Subpart B, § 6211(a)-(h), 103 Stat. 2154; Nov. 5, 1990, P. L. 101-508, Title IV, Subtitle A, Part 1, § 4008(b)(1)-(3)(A), Part 3, § [4207]4027(a)(1)(A), (2), (3), (k)(3), 104 Stat. 1388-44, 1388-117, 1388-124.)